

Q A pain-free labor sounds perfect! Are there any drawbacks to having an epidural?

There are several reasons why you might decide against an epidural for your labor.

» **Restricted mobility:** you will be limited in your ability to move around because you won't have any strength in your legs and because you will need to be attached to a fetal heart monitor. Even so-called mobile epidurals are still limiting—talk to your doctor about what's available in your hospital and how much freedom you'll be able to have.

» **Longer labor:** you may experience a slower labor because the pain medication can make the muscles of your pelvis weaker and so less effective at turning your baby into a good position for birth. This also slightly increases your chances of needing an assisted (instrumental) delivery (see p.222).

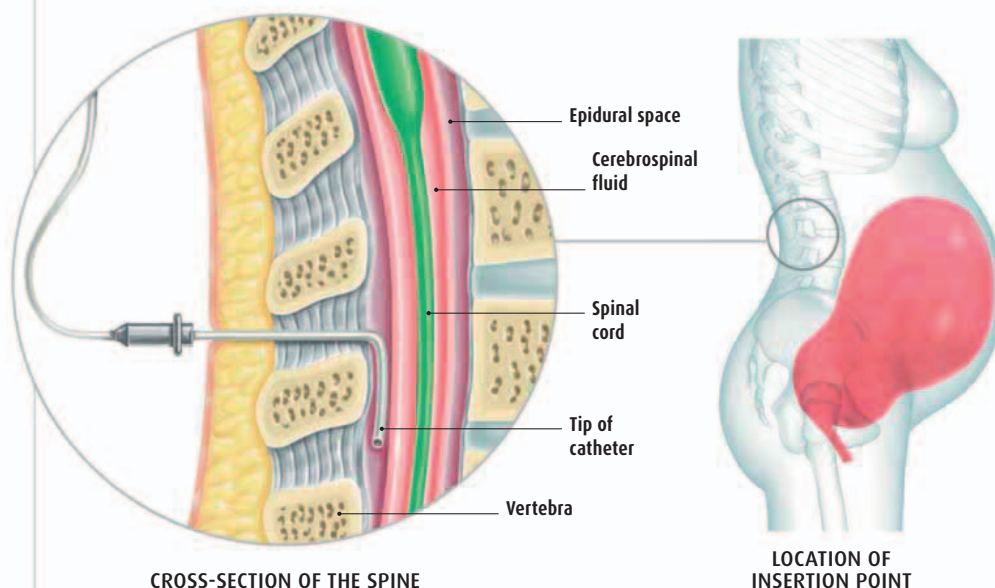
» **Increased odds of intervention:** there is a slightly increased risk of assisted delivery for women who have an epidural. Your doctor is likely to delay your pushing in second stage labor by up to an hour to reduce the risk of your needing an assisted delivery. However, you will need to deliver your baby within four hours of the start of the second stage, otherwise your doctor will recommend a C-section.

» **Limited effectiveness:** even if it has been effective for the first stage of labor, an epidural doesn't always work against the pain of the second stage of labor, when there is localized sharp pain as the baby emerges.

» **Risk of complications:** while most doctors agree there are no risks to your baby, epidural carries certain risks for you (see left).

INSERTING INTO THE EPIDURAL SPACE

The epidural space lies around your spinal cord, up and down its whole length. When you are given an epidural, a fine catheter is inserted directly into this space in order to administer drugs.



A mixture of **anesthetic** and **narcotic** injected into the spine **blocks the pain messages** from the uterus to the brain.

Q How long does an epidural take and how long does it last?

An epidural takes around 10 minutes to set up and around 15–20 minutes to become fully effective, and will last for as long as the anesthetic is topped off. Some hospitals have pump systems that allow you to control how much anesthetic you receive and when, leaving you to judge how much of your labor you want to be able to feel and when. Once you stop receiving the anesthetic, the effects should wear off and you should regain the feeling in your legs within around two hours.

Q I'm worried that an epidural might damage my spine. What are the risks and side effects?

Permanent damage to your spine following an epidural is very rare—occurring in between 1 in 80,000 and 1 in 320,000 cases, depending upon the source of the statistics.

If you experience any significant discomfort or pain in your epidural site when the needle is going in or immediately after it has been withdrawn, let your anesthesiologist know so that he or she can reposition the catheter if necessary. It is normal to feel a little achy in your back after an epidural, but if you have severe pain, tell your doctor.

Between 1 and 5 percent of epidurals may lead to a “dural tap” in which the epidural needle makes a small puncture causing cerebrospinal fluid to leak from around your spinal cord. The first symptom is often a bad headache. However, just like a nick anywhere else on your body, the wound will heal and the leak will stop on its own—all you will need to do is rest. Occasionally your anesthesiologist may want to plug the hole with a little of your own blood—almost like creating a scab to give the tissues beneath time to heal.

Other complications of an epidural include feelings of nausea and itchy skin, both of which are easily treated and will pass, and low blood pressure (you will probably be given medication intravenously to help prevent this side effect).



Did you know...

If you choose to have an opioid analgesic you will probably also be given an antiemetic—a drug that limits the side effects of nausea associated with these methods of pain relief.